

Respiratory R-09 Toxic Gas Inhalation BLS

Revised 3/1/2000

PRIORITIES:

- ABCs
- Determine degree of physiologic distress:
- Maintain airway, provide oxygen and ventilatory support;
- Assure an advanced life support response;
- Field treatment limited; CODE 3 transport is indicated for severe respiratory distress.

Toxic Gas Inhalation

Respiratory distress caused by inhalation of toxic gases by history. Suspect carbon monoxide with history of fire in an enclosed space; symptoms of headache, dizziness which may be associated with cherry-red coloration of mucus membranes (late sign).

1. Remove patient from toxic environment with attention to safety of rescue personnel;
2. Insure airway is clear and assist ventilation when indicated;
3. OXYGEN – 10 liters/minute or higher. If Carbon Monoxide suspected, consider 100% oxygen via non-rebreathing mask;
4. Assist advanced life support personnel with patient packaging and movement to ambulance.

Respiratory R-09 Toxic Gas Inhalation

PRIORITIES ➤ ABCs ➤ Determine degree of physiologic distress ➤ Evaluate respiratory rate, use of accessory muscles, cyanosis, ventilatory effort, level of consciousness ➤ Maintain airway, provide oxygen and ventilatory support PRN ➤ Early transport/ Hospital notification (after initiating therapy, if appropriate) ➤ EARLY CONTACT OF RECEIVING HOSPITAL.	MARCH 2007
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➤ Stabilize airway; ➤ Remove patient from toxic environment with attention to safety of rescue personnel ➤ OXYGEN – non-rebreather mask; assist ventilations if necessary for SEVERE respiratory distress, cyanosis, accessory muscle use and/or abnormal skin signs. ➤ Cardiac monitor ➤ IV access TKO; ➤ If WHEEZING, ALBUTEROL 2.5 mg by HHN or BVM; may repeat. x 2. ➤ BASE PHYSICIAN CONTACT FOR ADDITIONAL DRUG THERAPY.

DISRUPTED COMMUNICATIONS
In the event that a Solano County EMT-P is UNABLE to make physician contact for orders, the paramedic MAY NOT utilize those areas of the protocol needing physician direction and MUST TRANSPORT IMMEDIATELY TO THE CLOSEST FACILITY.